

Complications and long-term results of the Nissen fundoplication.

PMID: 8466868 | Indexed in PubMed

The Nissen fundoplication is the operative therapy of choice in children with symptomatic gastroesophageal reflux. In recent years several reports on specific complications of the Nissen fundoplication have been published. This report summarizes the complications in 45 patients on whom a Nissen fundoplication was performed in our clinic between 1979 and 1989. 18 patients (40%) showed early or late complications. In 10 patients (22%) reoperation was necessary. However, at follow-up with a mean period of 4 1/2 years after surgery, 36 patients (92%) showed good results without any residual symptoms. The Nissen fundoplication had in our hands a rather high rate of complications. However, late results were excellent and our healing rate (92%) was higher than reported in other series with different techniques.

Dumping syndrome following nissen fundoplication in an adult patient diagnosed by continuous online 13C/12C monitoring of ¹³C-Octanoic acid breath test "a case report".

PMID: 21929798 | Indexed in PubMed

Nissen Fundoplication is a common surgical procedure performed in treating gastroesophageal reflux disease (GERD). Complications include dysphagia, gastric hypersensitivity, abnormal gastric motility, gas bloat syndrome and GERD relapse. Dumping syndrome may occur when a large volume of gastric content is delivered to the duodenum or jejunum, resulting in both gastrointestinal and vasomotor symptoms. Occasionally, dumping syndrome may be a complication in patients that have undergone nissen Fundoplication, especially in adults. The BreathID® continuous online 13C-Octanoic acid breath test detects variations of less than 1/100,000 in the 13CO₂/12CO₂ ratio in exhaled air. We report a case of a 38 year old male who was admitted and diagnosed with dumping syndrome following nissen Fundoplication, who was diagnosed using the BreathID® continuous online 13C-Octanoic acid breath test. Early performance of a gastric emptying rate breath test in symptomatic patients, following upper GI tract surgery may help in the prediction or diagnosis of nissen Fundoplication complications such as dumping syndrome.

Laparoscopic Nissen fundoplication to treat gastroesophageal reflux.

PMID: 7778902 | Indexed in PubMed

Laparoscopic Nissen fundoplication is an effective treatment for gastroesophageal reflux. The procedure involves wrapping the gastric fundus around the lower end of the esophagus to augment the lower esophageal sphincter. The procedure offers several advantages over open laparotomy, including smaller abdominal incisions, reduced postoperative discomfort, and shortened postoperative recovery and hospital stays. The laparoscopic approach also reduces the length of time a patient has an ileus.

Open Nissen fundoplication.

PMID: 7583026 | Indexed in PubMed

The Nissen fundoplication currently is the most commonly used antireflux operation. It may be performed using a transabdominal or a transthoracic approach. Early postoperative complications are not infrequent and include dysphagia and the "gas bloat" syndrome. Excellent or good long-term results are obtained in more than 85% of patients.

Laparoscopic Nissen fundoplication: a technique for the easy and precise manufacture of a true fundoplication.

PMID: 10741453 | Indexed in PubMed

Passing the stomach behind the esophagus during laparoscopic Nissen fundoplication is a common source of frustration for the laparoscopic surgeon. It often leads to an incorrect formation of the fundoplication, resulting in a wrapping or twisting of the fundus around the distal esophagus. The correct technique should result in the distal esophagus being enveloped inside the fundus without distorting the orientation of the greater curve. We have developed an easy, precise, and reproducible technique to perform this maneuver. The steps for performance of this maneuver are described.

Single center prospective randomized trial of laparoscopic Nissen versus anterior 90 degrees fundoplication.

PMID: 16713542 | Indexed in PubMed

Although Nissen fundoplication is a very effective treatment for gastroesophageal reflux, it is associated with a small incidence of troublesome postoperative side effects. To prevent this, progressive modification of surgical techniques has led to the development of an anterior 90 degrees fundoplication. We undertook a prospective randomized trial to compare this procedure with Nissen fundoplication to determine whether it would achieve a better clinical outcome. Patients presenting to a single center for primary laparoscopic antireflux surgery were randomized to undergo either an anterior 90 degrees fundoplication (n = 40) or a Nissen fundoplication without division of the short gastric vessels (n = 39). Clinical questionnaires were used to assess outcome at 1 month, 3-6 months, and 12 months. Both patients and the clinical interviewer were masked as to which procedure was performed. Follow-up with endoscopy, esophageal manometry, and pH monitoring was also undertaken. Operating time was similar for the two procedures (60 minutes for anterior vs. 55 minutes for Nissen fundoplication). Early postoperative complications were more common after Nissen fundoplication (18% vs. 5%). Two patients underwent laparoscopic reoperation for recurrent reflux after anterior 90 degrees fundoplication, and four underwent laparoscopic reoperation after Nissen fundoplication (dysphagia, 3 patients; acute hiatus hernia, 1 patient). One year after surgery, dysphagia and other wind-related side effects were less common after anterior 90 degrees fundoplication. Control of reflux symptoms and satisfaction with the overall outcome was similar for the two procedures. Anterior 90 degrees fundoplication is followed by fewer side effects than Nissen fundoplication. This advantage is offset by a greater likelihood of reflux recurrence. However, this does not diminish patient satisfaction.

Laparoscopic Nissen fundoplication.

PMID: 7640948 | Indexed in PubMed

Most patients with gastroesophageal reflux disease (GERD) can be treated effectively with medical therapy; however, in patients with severe GERD who are unresponsive to medical therapy, the lower esophageal sphincter (LES) is often found to be mechanically incompetent. Surgical therapy, which improves the LES antireflux barrier, may then be a good option. A very effective and popular antireflux procedure is the Nissen fundoplication, which can be safely done via the laparoscopic route. Preoperative evaluation should include contrast radiography, esophagoduodenoscopy (EGD) with biopsies, esophageal manometry, and 24-hour pH monitoring. Indications for surgery include failure or inability to continue on medical therapy, GERD-related respiratory symptoms, and severe complications of GERD, such as ulceration, stricture, and Barrett's esophagus. A short, loose Nissen fundoplication is ideal for patients with normal esophageal body motility. Operative complications are infrequent, and they include gastric perforation, bleeding, and pneumothorax. Following the laparoscopic approach, nearly all patients can leave the hospital on the first or second postoperative day. Follow-up esophageal manometry and 24-hour pH monitoring show the same good long-term results as seen after open Nissen fundoplication. Laparoscopic Nissen fundoplication can be performed safely and effectively with all of the advantages of a minimally invasive approach.

Laparoscopic Nissen fundoplication for the treatment of gastroesophageal reflux disease.

PMID: 8304651 | Indexed in PubMed

The Nissen fundoplication is the most common operative approach to the correction of gastroesophageal reflux disease (GERD) in the United States. This report describes our success in performing this anti-reflux procedure laparoscopically in 28 patients with symptomatic GERD refractory to conventional medical therapy. Our laparoscopic technique does not differ significantly from the traditional, open Nissen fundoplication. After surgery, all patients reported symptomatic relief, and none required medication for the control of reflux symptoms. Oral feedings were begun on the first postoperative day, and patients were typically discharged on the second day after surgery. All but two patients resumed normal eating within an average of 26 days. Two patients experienced longer term postoperative dysphagia, including one who had undergone a highly selective vagotomy concurrent with the antireflux surgery.

Laparoscopic Nissen fundoplication.

PMID: 1341529 | Indexed in PubMed

Laparoscopic Nissen fundoplication represents a minimally invasive surgical approach to symptomatic gastroesophageal reflux and may offer patients an attractive alternative to indefinite medical therapy. Fourteen patients with persistent gastroesophageal reflux and two with severe achalasia underwent a laparoscopically guided 360-degree or Nissen fundoplication. Both individuals with achalasia also had an extended distal esophagomyotomy. Two patients required conversion to open laparotomy because of difficulties in defining the anatomy of the posterior esophagus. All 16 patients reported complete relief of

their symptoms. The average length of hospital stay was four days. These early results of laparoscopic Nissen fundoplication are encouraging and hopefully will stimulate further experience in this area.

Fundoplication.

PMID: 29127961 | Indexed in PubMed

Gastroesophageal reflux disease (GERD) is a common condition in infants. Symptoms from pathologic GERD include regurgitation, irritability when feeding, failure to thrive, and respiratory problems. Treatment typically starts with dietary modifications and postural changes. Antireflux medications may then be added. Indications for operative management in neonates and infants include poor weight gain, failure to thrive, acute life-threatening events, and continued respiratory symptoms. Laparoscopic Nissen fundoplication has become the standard of care for surgical treatment of children with GERD. In this procedure, the fundus of the stomach is wrapped 360° posteriorly around the lower esophagus.
